Sincerely,

GENERATIONS VISITING PRACTITIONERS, LLC
4400 NW Loop 410 Suite 115
SAN ANTONIO, TX 78229
P:210-598-8035
F:210-888-1703

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Fax

To: TURNER, JOYCE	**From:** Pam Health at Home - Texas
Fax: 12108881703	**Pages:** 8
Phone:	**Date:** 02-23-2026 4:48 PM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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Order No:485-25398

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
98005919101	02/05/2026	02/05/2026 - 04/05/2026	9461	262009456

6. Patient's Name and Address
STEPHENS, BILLY R (9461)
7524 WILD EAGLE
San Antonio, TX 78255
Phone: (425) 344-6291

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 04/10/1935 | **9. Sex:** ☒ M ☐ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Ergocalciferol (vitamin D2) 1,250 mcg (50,000 unit) capsule [Amount Of/ Dosage Form: 0 Capsule]	1 CAP	weekly	oral	LS
Lactulose 10 gram/15 mL oral solution [Amount Of/ Dosage Form: 0 Milliliter]	10ML	daily	oral	LS
Trazodone 150 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1 TAB	at bedtime	oral	LS

11. ICD-9 / 10 CM **Principal Diagnosis**

Code	Code Description	Date	O/E
Z46.6	Encounter for fitting and adjustment of urinary device		

12. ICD-9 / 10 CM **Surgical Procedure**

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM **Other Pertinent Diagnosis**

Code	Code Description	Date	O/E
N40.1	Benign prostatic hyperplasia with lower urinary tract symptoms		
N39.46	Mixed incontinence		
L89.142	Pressure ulcer of left lower back, stage 2		
K92.2	Gastrointestinal hemorrhage, unspecified		
K59.01	Slow transit constipation		
F43.21	Adjustment disorder with depressed mood		
F51.01	Primary insomnia		
R22.0	Localized swelling, mass and lump, head		
E55.9	Vitamin D deficiency, unspecified		
R73.03	Prediabetes		
Z96.0	Presence of urogenital implants		

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14. DME and Supplies:
DME: Dressing Supplies, Elevated Toilet Seat, Grab Bars, Tub/Shower Bench, Walker, Wound Supplies
Supplies: Foley Cath.Ins.Ty, St, 10Cc, Urinary Drainbag W/Hook, 2000Cc

15. Safety Measures:
911 Emergency / Protocol, Assistance with IADLs, Fall / Safety Precautions, Fall Precautions, Keep Pathways Clear and Safe, Safe in Movements, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
Regular,

17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☐ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☑ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☐ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☐ 1 - Oriented ☐ 2 - Comatose ☑ 3 - Forgetful ☐ 4 - Depressed ☑ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☑ 2- Guarded ☐ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
SN Frequency: 2W1, 1W2 Starting Week of 02/05/2026,PT Frequency: 1W3 Starting Week of 02/05/2026,

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28

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BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Weight: < 135 or > 145
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:

- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

CATHETER:

- SN to instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.
- SN to instruct patient/caregiver on daily foley care.

GENITOURINARY:

- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.

OTHER WOUNDS:(STASIS ,SKIN TEARS,EDEMA,RASHES,ETC):

- Clinician to assess wound and instruct/perform wound care as follows: RIGHT EAR AND TEMPLE: CLEAN WITH NORMAL SALINE AND PAT DRY LEAVE OPEN TO AIR.
  Home health to provide wound care supplies as ordered. Clinician to discontinue wound care when healed.

WOUND CARE:

- Clinician to assess wound, instruct/perform wound care as follows: CLEANSE WITH NS PAT DRY APPLY OPTIFOAM.
  Home health to provide wound care supplies as ordered. Clinician to instruct patient/caregiver on S/s of infection to report. Clinician to discontinue wound care when wound is healed.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

PT

PHYSICAL THERAPY:

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- Evaluation and Treatment
- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: Balance, coordination, and/or agility training
- Therapeutic Exercises: Strength training
- Gait Training: Even and uneven surfaces and/or natural terrain
- Transfer Training : Supine <>Sit, Sit <> Stand, Bed <> Chair, On/Off toilet or commode, In/Out shower or tub , Floor , car
- Transfer Training: Floor <> stand, floor <> chair, and/or floor <>bed
- Bed Mobility: Scoot/roll/bridge

Assessment Summary

THIS RN WAS ESCORTED TO THE PATIENTS ROOM BY STAFF MEMBER. PATIENT IS IN ASSISTED LIVING TYPE B FACILITY. PATIENT IS FOUND LAYING IN HOSPITAL BED. PATIENT IS VERY CONFUSED ON WHY THIS RN WAS PRESENT. THIS RN PREFORMED BIMS ASSESSMENT ON PATIENT TO WHICH HE SCORED A 5. THIS RN DISCUSSED HOME HEALTH WITH PATIENT AND REVIEWED ADMISSION AND POC WITH PATIENT AND AGREED TO POC AND SIGNED ADMISSION PAPERWORK. THIS RN ALSO GOT PATIENTS SON GARY TO SIGN AS PATIENT HAS LOW LEVEL OF COGNITION. PHYSICAL ASSESSMENT COMPLETED THIS VISIT. VS WNL, BREATH SOUNDS CLEAR THROUGHOUT, BOWEL SOUNDS ACTIVE X4, SKIN ASSESSMENT PREFORMED AND NOTED THAT PATIENT HAS AN OPEN LESION TO RIGHT EAR WITH NO ORDERS BUT HAS A DERMATOLOGIST APPOINTMENT NEXT WEEK, CLOSED LESION TO RIGHT TEMPLE, AND A STAGE 2 PRESSURE ULCER TO SPINE. OFFLOADING EDUCATION PROVIDED TO PATIENT AND FACILITY EMPLOYEE. EMPLOYEE STATES HE CAME FROM THE HOSPITAL WITH IT BUT WILL BE MORE DILIGENT ON REMINDING PATIENT TO OFFLOAD. PATIENT DOES HAVE A 16FR10CC FOLEY CATHETER THAT WE DO NOT HAVE ORDERS FOR THAT WAS PLACED IN THE HOSPITAL. PATIENT HAS A UROLOGIST APPOINTMENT NEXT WEEK AS WELL. CATHETER SITE IS CLEAN AND INTACT.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. STEPHENS, BILLY R is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)

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c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 2.03.2026 BY TURNER, JOYCE APRN and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 2.05.2026.

Risk of Hospitalization

Risks: Unintentional weight loss of a total of 10 pounds or more in past 12 months; Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Decline in mental, emotional, or behavioral status in the past 3 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;
Interventions: Referrals - SN,PT; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Fall Prevention Program
Risk Factors: Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); Confusion (M1710); Pressure Ulcer (M1300, M1302 & M1306); Inadequate Support Network (M 1100); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required; Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions

Advance Directive

POA

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted;
- Patient Chronic and Stable: No
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Guarded to achieve stated goals with skilled interventions and patient's compliance with plan of care.

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Discharge Plan
Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

OTHER WOUNDS:(STASIS, SKIN TEARS, EDEMA,RASHES,ETC): - LTG
- LTG-Patient/caregiver will be able to identify/ recall S/S of infection to report by end of episode.

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

CATHETER: - LTG
- LTG-Patient/caregiver will demonstrate independence in catheter care by end of episode.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

PT

FUNCTIONAL MOBILITY: - STG
- STG-Pt will demonstrate improved functional mobility with getting in/out of bed mobility and sit to stand transfers with independence and decrease caregiver burden.

MOBILITY: - LTG
- LTG-Pt will improve Tinetti test score to 24/28 to enhance balance and stability during activities of daily living.

MOBILITY/GAIT: - LTG
- LTG-Patient will ambulate on level surface a continuous distance of 200ft using 2WW with Supervision to improve safety at home and enhance ability to walk to bathroom from living room or bedroom.

STRENGTH: - LTG
- LTG-Patients will be independent with BUE/BLE home exercise programs in order to maintain gains made in-home therapy.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/05/2026
Signed By: Castro, Paige - RN on 02/05/2026 10:52 AM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

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24. Physician's Name and Address
Joyce Turner FNP
~~2040 BABCOCK RD STE 304~~ 4400 NW Loop 410 Ste 115
SAN ANTONIO, TX 78229,NPI: 1851502629, License#: AP116603,Phone: (210) 598-8035, Fax: (210) 888-1703

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:
02/26/2026
Joyce Turner APRN, FNP-BC

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.